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BEREAVEMENT RESEARCH: CONTEMPORARY PERSPECTIVES

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Understanding of the bereavement experience has broadened and deepened in recent decades. Earlier assumptions have been put to empirical test; new and creative research designs have been introduced; innovative statistical techniques have been applied; and there have been novel attempts at theoretical integration, lively disputes about controversial issues, and significant developments in intervention programming. We have come to understand as well the influence of dramatic societal change and the ways in which events of national and international scope have altered the circumstances of bereavement to which people must adapt, affecting how they grieve personally and mourn publicly. Bereavement research continues to reflect the involvement of many clinical and academic disciplines and investigators from many countries and cultures.

These developments, then, suggest the timeliness of the present volume. With the turn of the century not long behind us, it is appropriate to take stock, to sort through and attempt an integration of the new knowledge base, and to come to some consensus regarding priorities and directions for future research efforts.

Our objective for this volume was to continue to provide an up-to-date, state-of-the-art account of bereavement research, as we have attempted to

provide in our previous *Handbooks* on the topic of bereavement (M. Stroebe, Hansson, Stroebe, & Schut, 2001; M. Stroebe, Stroebe, & Hansson, 1993). However, in the current volume we have tried to relate bereavement research more explicitly to contemporary societal and practice issues than was done in earlier volumes. In this context, it is important to emphasize that this volume is not a sourcebook for practitioners seeking practical tips. It is instead intended to provide—insofar as this is possible in a single volume—a comprehensive overview of research in relationship to practice, identifying developments in the field, discussing contemporary controversial issues, encouraging debate about them, and proposing research and implementation agendas for the future.

All the chapters in this volume are new, except for chapter 7 (which we discuss in more detail later in this chapter). Chapter 7 is an update and considerable expansion of a similar chapter in the previous *Handbooks*. The remaining chapters all address topics and perspectives that complement and build on the contents of previous *Handbooks*. Chapters were sought from many new authors on emerging, innovative topics and lines of research on bereavement.

In the following sections of this chapter, we have three goals. First, for the reader's convenience, we define and discuss fundamental concepts in the bereavement field, ones that are frequently used by the contributors to this volume. We then trace the major historical shifts that have taken place in scholarly thinking about bereavement to set the stage for the contemporary developments in research and practice. Finally, we introduce the reader to the structure and scope of the volume itself and outline the unique contributions of the individual chapters.

BEREAVEMENT RESEARCH: DEFINITIONS OF FUNDAMENTAL CONCEPTS

In the paragraphs that follow, we provide definitions of the basic concepts and manifestations associated with bereavement. These definitions are followed with reasonable consistency by the authors in this book and across the bereavement research field in general (cf. Archer, 1999; Averill, 1968; Parkes, 1996; M. Stroebe et al., 2001). Despite the concordance, it will become evident to the reader that none of these definitions are actually cut and dried and that thoughtful consideration and ongoing discussion about the basic concepts are still necessary.

Bereavement, Grief, and Mourning

Bereavement is the term used to denote the objective situation of having lost someone significant through death. This naturally leads to the question of

how to define *someone significant*. The category is generally taken to include personal losses experienced across the life span: the deaths of parents, siblings, partners, friends, and—against the expectations of parents—one’s own child. Bereavement is associated with intense distress for most people. This is one of the main reasons for the burgeoning body of research on the topic; a major concern is to understand and try to relieve the suffering of bereaved people.

Grief is the term applied to the primarily emotional (affective) reaction to the loss of a loved one through death. It is a normal, natural reaction to loss: As Archer (1999) expressed it, “Grief . . . is the cost we pay for being able to love in the way we do” (p. 5). Although grief is understood to be primarily a negative affective reaction, it also incorporates diverse psychological (cognitive, social–behavioral) and physical (physiological–somatic) manifestations. It is a complex syndrome, within which a variety of symptoms may be apparent. The symptom picture, however, is understood to vary considerably from one bereaved person to another, from one culture to another, and across the course of time even for a single grieving individual (and again—across the course of time—between cultures).

As an illustration of differences in grief symptoms, consider the following examples (cf. Rosenblatt & Wallace, 2005): One person’s grief may be dominated by intense feelings of loneliness, another’s by anger at being abandoned by the loved one. Bereaved people from one ethnic group may experience (or express) feelings of grief that vary in nature and intensity from those experienced or expressed by other ethnic groups. One person may show few signs of grief early on but intense reactions later, whereas another person may manifest the opposite duration-related effects.

Mourning, in our view, is a term that should be distinguished from grief.¹ We consider *mourning* to refer to the public display of grief, the social expressions or acts expressive of grief that are shaped by the (often religious) beliefs and practices of a given society or cultural group. For a fascinating review of culture contrasts in mourning customs and rituals, we refer the reader to a volume by Parkes, Laungani, and Young (1997), whose descriptions of rituals and beliefs surrounding death range from those that are permeated and guided by Buddhist religious doctrine among traditional Tibetans (Gielen, 1997) to those influenced by secular, rational, humanistic philosophy in the West (Walter, 1997).

Although we define them separately, mourning and grief have at times been difficult to distinguish (an issue that is raised again by Rosenblatt in chap. 10 of this volume). For example, it may be unclear whether an overt expression of distress is a reflection of an emotional, personal reaction or

¹*Mourning* is sometimes taken to mean the emotional reaction to loss, which we have defined as *grief*. Such usage is followed particularly by researchers and practitioners adhering to the psychoanalytic tradition, who sometimes use the terms *grief* and *mourning* interchangeably.

whether the bereaved individual is following a societal norm to express emotion (e.g., in the case of spontaneous vs. ritual crying). Grief may influence mourning, and mourning may equally influence feelings of grief. Such possibilities are illustrated in a recent account by Rubin and Yasien-Esmael (2004) of Muslim traditions in Israel, which are also likely to contrast with the experience of grief and mourning among those from other cultural and religious backgrounds:

The extent to which the bereaved may remember and long for the deceased in Islamic tradition at the level of private psychological experience is not clearly set forth. It would seem that the bereaved is allowed to experience the complex of memories and emotions vis-à-vis the deceased from a generally positive psychological valence, and with a sense that the beloved is with God. . . . Mourning practices and their place in the psychological, communal, and religious life of Muslims within Israel today leave room for ongoing personal attachments to the deceased. The religious and behavioral focus is on acceptance of fate, the reality of the loss, and the primacy of following religious practices in the relationship to God, even at one's moments of great pain and upheaval. (pp. 158–159)

According to this account, then, rituals among members of this Muslim community positively affect feelings of grief over the deceased person.

Normal and Complicated Grief

Researchers are generally hesitant to define *normal* grief versus *complicated* grief,² given that grief is a complex emotional syndrome, encompassing myriad reactions, durational changes, and cultural differences. In accordance with our definition of grief, *normal* grief could be defined as an emotional reaction to bereavement, falling within expected norms, given the circumstances and implications of the death, with respect to time course and/or intensity of symptoms. This definition, however, raises further questions; for example, what are the expected norms? On what basis should loss experiences be classified with respect to circumstances and implications for the bereaved? What are people's expectations with respect to duration of symptoms, and how can a cutoff point for intensity be established?

Additional difficulties arise in trying to characterize complicated grief. Complicated grief is not a single syndrome; it is subject to cultural variation; it is sometimes difficult to differentiate from related disorders (e.g., depression, anxiety disorders, posttraumatic stress disorder) and, despite major contemporary efforts, researchers have yet to reach agreement regarding a definitive

²Other terms also are frequently used, for example, *pathological*, *traumatic*, *unresolved*, *prolonged*.

set of diagnostic criteria (or even whether diagnostic criteria are necessary and useful; see Parkes, 2005).

In this context, we can begin with our previous definition of *complicated* grief (M. Stroebe et al., 2001), namely, a deviation from the (cultural) norm (i.e., that could be expected to pertain, according to the extremity of the particular bereavement event) in the time course or intensity of specific or general symptoms of grief. However, such a definition fails to take dysfunction into account.³ Intensity implies dysfunction, but daily functioning in various spheres of life following bereavement should probably be made more explicit, especially because this dimension is clinically relevant. The *Diagnostic and Statistical Manual of Mental Disorders* (4th ed.; DSM-IV; American Psychiatric Association, 1994) criteria for clinical significance (for mental disorders) usually include the specification that the condition “causes clinically significant distress or impairment in social, occupational, or other important areas of functioning” (p. 7). Thus, the roles of function and performance should also be taken into account in defining complicated grief. A two-pronged extension of our definition of complicated grief thus seems warranted: a clinically significant deviation from the (cultural) norm (i.e., that could be expected to pertain, according to the extremity of the particular bereavement event) in either (a) the time course or intensity of specific or general symptoms of grief and/or (b) the level of impairment in social, occupational, or other important areas of functioning.

Types of Complicated Grief

Various subtypes of complicated grief have received extensive attention in the clinical and research literatures. For example, *chronic* grief, which is similar to *prolonged* grief (see chap. 8, this volume), is characterized by long-lasting presence of symptoms associated with intense grief. *Delayed*, *inhibited*, or *absent* grief occurs when an individual shows little or no sign of grieving early on in bereavement but, in the cases of delayed and inhibited grief, does so intensely at a later time (again, pathology is not always indicated by early absence of symptoms). Absent grief can be linked with delayed and inhibited grief because of conceptual overlap: Although symptoms are apparently absent, they too may be manifested at some later time, or difficulties may be apparent in terms of some other grief-related debility or disorder, suggesting that the absence was indeed problematic. Like delayed and inhibited grief, absent grief is characterized by the nonappearance of overt symptoms typical of grief, with the continuation of life seemingly unaffected by the bereavement. As we suggested earlier, absent grief may also be problematic if the person man-

³We are grateful to E. Zech (personal communication, July 2006) for drawing attention to this point.

ifests symptoms that are not typical of grief (i.e., that take the place of grief symptoms); however, we need to emphasize that the absence of grief symptoms without the presence of other symptoms gives rise to caution in interpreting lack of apparent grief in terms of problems. Chronic (or prolonged) grief has been generally well accepted as a pathological category, whereas delayed, inhibited, and absent grief remain the subject of much debate.

In presenting these categories it is important to note that only a minority of bereaved persons suffer from complicated forms of grief and will need the professional help of counselors or therapists. Most bereavements involve no pathological indications. The majority of bereaved persons will undergo a period of intense suffering and some dysfunction but will be able to deal with their bereavement without professional referral.

Grief Counseling and Grief Therapy

Many different types of supportive intervention are available for bereaved persons in Western societies, and these are gradually extending to other cultures, notably in the wake of recent natural disasters. It is difficult to categorize the various programs meaningfully, but Worden (1982; see also Worden, 1991, 2001) has provided a useful distinction between grief counseling and grief therapy. *Grief counseling* refers to the facilitation through counseling of the process (tasks) of normal, uncomplicated grieving to alleviate suffering and help bereaved individuals to adjust well, within a reasonable time. An example is the Widow-to-Widow mutual help program, developed in the United States by Silverman (e.g., Silverman, 1986). *Grief therapy* refers to specialized techniques of intervention that guide an abnormal or complicated grief reaction (e.g., chronic grief) toward a normal coping process. Of course, in practice it may be difficult to make the distinction between these two types of intervention.

BEREAVEMENT RESEARCH ACROSS THE 20TH CENTURY

In this section, we provide an overview of the issues and concerns that have occupied scientists across the 20th century to place contemporary bereavement theory and research in a broader perspective. We focus on a few of the major researchers who have shaped the scientific study of bereavement (more comprehensive accounts can be found in Archer, 1999; Parkes, 1972, 1986, 1996; W. Stroebe & Stroebe, 1987).

Early Developments

Nearly a century ago, Freud's (1917/1957) classic paper "Mourning and Melancholia" provided the first systematic analysis of bereavement. The

paper discussed reactions to the death of a loved one from Freud's psychoanalytic viewpoint, and his theoretical ideas became highly influential in shaping subsequent understanding of healthy and unhealthy coping with bereavement. His emphasis on the necessity for individuals to do grief work in coming to terms with a loss was to be taken up and elaborated for many decades to come—indeed, right up to the present. In subsequent decades, for example, both Deutsch (1937) and Klein (e.g., 1940) began to address, also within the psychoanalytic tradition, issues of complicated grief, with the former analyzing the phenomenon of absent grief and the latter the relationship of grieving to manic-depressive (i.e., bipolar) states. As already indicated, the demarcation of complicated grief is a topic that continues to occupy bereavement researchers today.

The early theoretical contributions to bereavement research reflected a psychoanalytic tradition. However, with the publication of Lindemann's (1944) article, titled "Symptomatology and Management of Acute Grief," researchers began to conduct empirical studies of grief and its consequences. The range of symptoms that Lindemann found to be associated with grief are still today reflected in assessments of bereaved persons. Even earlier, epidemiological studies in the 19th century, notably those by Durkheim (1951/1987) and Farr (1858/1975), had begun to explore the consequences of bereavement for mortality. This work was advanced in the middle of the 20th century by Kraus and Lilienfeld (1959), who showed that the mortality risk of the widowed was consistently higher than for married counterparts of the same age and sex. Bereavement-related mortality rates continue to hold the attention of contemporary researchers.

Mid 20th Century Onward

During the 1950s, researchers began a more systematic documentation of the manifestations and duration of grief. The work of Clayton and colleagues (e.g., Clayton, 1979) and Maddison and colleagues (e.g., Maddison & Viola, 1968; Maddison & Walker, 1967) made prominent contributions to this, and an important conceptual article by Averill (1968) brought focus to bereavement as a field of research by delineating central issues and concepts. Researchers began to map the mental and physical vulnerabilities of grieving persons with an eye to providing the right type of care to those who most needed it. Parkes's (1972) *Bereavement: Studies of Grief in Adult Life* consolidated much of the research that had been conducted up to that point. This classic book has continued to shape the field, with new editions appearing at regular intervals (Parkes, 1986, 1996).

During these years, with few exceptions, the focus centered on the grieving individual, the perspective being intrapersonal rather than interpersonal. Vachon's pioneering work (e.g., Vachon et al., 1982), however, extended the

investigation to interpersonal issues to include social risk factors, the effectiveness of intervention programs for the bereaved, and the impact of bereavement on social networks (and vice versa). A feature of this historical period was that empirical research (despite the psychoanalytic tradition) was still largely issue rather than theory driven. For example, a major practical issue driving much research from the mid 20th century onward concerned the identification of high-risk categories of bereaved persons; given the vast individual differences in patterns of adjustment to the loss of a loved one, could researchers establish which subgroups of bereaved persons were most vulnerable, and to what specific health consequences?

These developments, and advances in the broader fields of stress and trauma, then led to further explorations of the nature of complicated grief. Investigations by Parkes (1965) and Parkes and Weiss (1983) resulted in a highly influential classification of the complications of grief based on a risk factor perspective. Jacobs (1993) subsequently made the link between complicated grief and the *DSM* system (cf. *DSM-IV*; American Psychiatric Association, 1994), suggesting a classification of so-called "pathologic grief" for potential inclusion in future editions of the *DSM*. In this period, also, Bowlby (1980) examined chronic and absent forms of grieving from his attachment theory perspective (see also Raphael, 1983).

Also by the later decades of the 20th century, stage models for adaptation to grief (e.g., Bowlby, 1980) and task models (e.g., Worden, 1982) had begun to shape researchers' understanding of the course of grieving. These were welcomed as guidelines by those counseling the bereaved. Unfortunately, there have been misinterpretations, particularly about Bowlby's (1980) stage model. Bowlby did not conceive of the postulated stages of grief as prescriptive with respect to where a given bereaved person should be at any particular duration of bereavement (although it is probably true to say that other stage theorists actually did so). There is much overlap in symptoms across proposed stages. Not all bereaved people show indications of going through phases or tasks, or in the sequence suggested, and there is typically no clear end point; bereaved people—at least in Western culture—do not simply recover or "get over" their loss and return to normal; there is no resolution or completion per se; instead, they adapt, adjust, and are to some degree changed forever (Weiss, 1993).

In recent decades, research has become increasingly theory guided; cognitive stress theory and attachment theory in particular have influenced research psychologists in the field, and attention has increasingly focused on the complexities of bereavement experience. Gradually, too, research designs, methods, and statistical techniques have become more sophisticated. Prospective, multivariate designs are now being used to address more finely grained processes underlying the manifestations of grief. Laboratory studies are beginning to become more frequent as well.

The Turn of the Century

Our *Handbook of Bereavement Research* (M. Stroebe et al., 2001) represented the state of affairs at the turn of the century, providing comprehensive reviews of theoretical and empirical advances in the field. That volume was organized around the three themes of “consequences,” “coping,” and “care.” These three themes merit further investigation, and each one is well represented across the chapters of this new *Handbook*.

Consider the first theme, “consequences”: By now it is well-known that the costs of bereavement in terms of mental and physical health can be extreme, and biological links have become much better established. Much still needs to be learned, however, about cultural differences in reactions (in terms of the relative frequency, intensity, duration, and even the nature and meaning of symptomatology). Researchers and practitioners also need to better understand resilience and to identify the personal and circumstantial factors that enable some persons to cope with a loss much better than others. There is a need as well to broaden the focus beyond health consequences (e.g., to social implications and to different types of dysfunction and debility).

Coping refers to processes, strategies, or styles of managing (reducing, mastering, tolerating) the situation in which bereavement places the individual. Grief work would be an example of coping. We noted elsewhere (M. Stroebe et al., 2001) that there is a thin line between the concepts of coping and symptomatology, but it remains important for theoretical and practical reasons to try to tease them apart (e.g., to retain clarity in the design of research by distinguishing process from outcome variables). Furthermore, although much interest in coping stems from a concern to identify more versus less successful strategies, the way one copes does not fully determine the outcome. Other factors, such as personality variables or circumstances of the death, may explain more variance in outcome than coping, and empirical studies involving multiple variables, instead of single-factor investigations, are needed (M. Stroebe, Folkman, Hansson, & Schut, 2006). Researchers also still need to identify the coping strategies that are effective (and for whom) and to better understand precisely what has to be coped with.

The third theme, “care,” focused on

ways that bereavement research can inform health care professionals and members of informal networks surrounding bereaved persons . . . includ[ing] knowledge about the nature of complications in the grieving process, the types of psychological and pharmacological interventions that are now available, and assessment of the effectiveness of such programs. (M. Stroebe et al., 2001, p. 10)

Bereavement researchers, ourselves included, are typically anxious to emphasize that grief is a normal reaction to the death of a loved one and that most

people manage to come to terms with their loss over the course of time. Although associated for most people with a period of distress, it does not usually require the help of professional counselors or therapists (although if bereaved persons feel the need for intervention, it is important that assessment procedures be available to establish whether they could be helped). For a minority, though, mental or physical consequences can be extreme and sometimes persistent. Progress has been made over recent decades in designing and implementing a range of care services for bereaved persons. As the chapters of this volume show, researchers are continuing to grapple with the difficult task of establishing the efficacy of such programs, using controls in their designs that were lacking in many earlier studies.

Where does this leave us now? In the final chapter of the 2001 *Handbook*, we pinpointed core themes emerging from the preceding chapters, to suggest directions for future research (M. Stroebe et al., 2001). We also highlighted the major controversies in the field. In many ways, this current *Handbook* carries on where the last one left off, picking up new directions and neglected themes and furthering discussion of controversial issues.

SCOPE AND CONTENT OF THIS VOLUME

Before we describe the themes around which the chapters in this current volume are organized, it is important to say a few words about our particular theoretical orientation. Our view of bereavement research is based on several fundamental principles, some of which have probably already become evident. Perhaps the most fundamental is our belief that bereavement needs to be understood from a sound base of theoretically oriented and empirically derived knowledge and not purely on the basis of subjective descriptive accounts, not even those of skilled individuals. Although clinical reports are of considerable value in designing research and in understanding bereavement-related phenomena, we strive wherever possible to report methodologically stringent empirical research and to suggest ways that claims made in theoretical or clinical contexts can be put to rigorous test. Research should ultimately not only lead to deeper scientific understanding but also be practically useful; concern about the impact of loss on bereaved individuals and groups underlies the work of the contributors to this volume.

We advocate as well a broad scientific approach to bereavement. This volume includes the work of researchers from diverse disciplines, including medicine, public health, sociology, religion, gerontology, anthropology, social and clinical psychology, and so on. We view these different perspectives as integral to greater synthesis and deeper understanding of the bereavement experience. In this and previous *Handbooks* (e.g., M. Stroebe et al., 1993, 2001), we have also tried to include the work of scientists from different countries. So far, how-

ever, although fascinating studies are beginning to emerge from other countries, well-established programs producing sophisticated lines of research seem regrettably still pretty much limited to Western industrialized countries.

Finally, in this volume in particular, we have attempted to broaden and deepen our coverage of cultural and ethnic issues in bereavement. Cultural issues are a persisting concern, not least because of population trends in contemporary society. We need more comprehensive and differentiated knowledge about the ways that persons from very different cultures grieve and about the belief systems and mourning customs that shape and give meaning and expression to the grief experience. Yet cross-cultural investigation on an empirical level in the bereavement field is still in its infancy: It will take time for cross-cultural studies of bereavement to reach the level of maturity attained in other fields of investigation.

Turning now to the structure of this volume, the research perspectives covered here are organized around six themes. These make up the main parts of the book.

Contemporary Scientific Approaches and Issues

The chapters in Part II of this volume provide theoretical background and examine the major conceptual, methodological, and measurement issues in current-day bereavement research and provide a conceptual base for the remaining chapters in the book. At a general level, these chapters elucidate basic questions: What is grief? What theories best explain its manifestations? How have theoretical understandings changed over time, and as a function of societal systems? At a more specific level, the questions examine the following issues. How might further integration of theoretical perspectives prove useful? For example, can a theory of lost interpersonal relationships increase comprehension of who suffers most among bereaved persons? How can one actually measure grief and grieving, and how valid are researchers' attempts to do so across different ethnic and cultural groups?

In chapter 2, Robert S. Weiss probes the origins and nature of grief, tracing developments in the understanding of grief across the 20th century and exploring contemporary thinking on fundamental issues such as the course of grief and the nature and function of bonds with the deceased and mental representations of the deceased. Weiss illustrates, and places in perspective, the wealth of scholarship that has accumulated not only within the bereavement field but also in related fields, such as emotion, to provide scientific backing for contemporary formulations.

In chapter 3, John Archer provides a broad overview and critical commentary on theories that have been influential in past and current research, from evolutionary to social constructionist, and from analytic to holistic approaches. For example, he takes a critical look at research and theory on

grief work, evaluating the utility of the concept and available evidence that bereaved persons actually need to confront and work through their grief to come to terms with it.

In chapter 4, the last of the three general theoretical chapters in this part, Robert Kastenbaum poses the following question: How might patterns of grieving and mourning change as we move deeper into the 21st century and beyond? He draws on his considerable experience in the fields of death, dying, and bereavement to assess ways that grief and grieving may influence individuals in the 21st century's evolving societies. Of particular importance is his consideration of grief not only within the individual but also within the context of societal death systems; he examines the interaction among individual experiences, cultural circumstances, and social forces. He examines as well how changing circumstances of dying can influence the ways in which people grieve. He then considers the phenomenon of grieving not only from one's personal bereavement perspective but also in terms of the loss of the world as it had been experienced and understood.

In chapter 5, Mario Mikulincer and Phillip R. Shaver turn from general theoretical issues to a specific, highly influential theory in recent bereavement research. Within the bereavement field, attachment theory's influence has increased phenomenally over the past 2 decades (this was a major topic in the last *Handbook* as well; M. Stroebe et al., 2001). There are good reasons to argue that this type of research is critical; attachment theory is a relationship theory that focuses on reactions to separation from a significant person. It follows that this perspective should offer insights into what happens when an attachment figure dies. Mikulincer and Shaver have used innovative methods to examine underlying processes relating to attachment patterns. In chapter 5, they outline the attachment theoretical perspective and review findings from their own and others' studies.

Also within the general theme of lost relationships is the question of whether an ongoing attachment to the deceased person should be continued during the grieving process or be relinquished. This remains a source of raging debate in the field. There has been a shift in recent decades, from an early-20th-century emphasis on the need to break bonds with a deceased person in order to adjust to loss and go on with life toward an emphasis on the need to sustain such bonds. Most recently, researchers have begun to approach this controversy in empirical as well as theoretical ways, examining the links between continuing and relinquishing bonds and adapting to grief. In chapter 6, Nigel P. Field reviews the continuing-bonds research and describes his own perspective. He presents his empirical research that has examined ways that people continue bonds and implications this might have for adjustment.

The topic of measurement also needs to be addressed. In the last *Handbook* (M. Stroebe et al., 2001), Robert A. Neimeyer and Nancy S. Hogan crit-

ically reviewed available quantitative and qualitative measures used in the study of grief. Since then, however, new instruments have emerged, and new, innovative techniques have been developed to examine the complex experience of grief. In chapter 7, with coauthor Anna Laurie, Neimeyer and Hogan update and extend their earlier review of quantitative and qualitative research approaches, addressing the following fundamental question: To what extent has the assessment of adaptation to bereavement moved toward greater psychometric rigor and sensitivity in recent years? Of particular importance in this chapter is that they appraise strategies and problems of measurement from a cross-cultural perspective: How can measures be adapted for use in various cultural and subcultural settings?

Contemporary Societal and Practical Concerns

The chapters in Part III explore contemporary practice and societal concerns associated with bereavement. The reader will note in this part considerable controversy surrounding a number of core questions; for example, how does one conceptualize those aspects of grief that require professional intervention? How does one ensure that bereavements among diverse components of a society are addressed with cultural competence? How does one ensure support for bereaved persons who may be isolated or whose grief may have become disenfranchised?

Perhaps the most important (and controversial) issue currently facing bereavement researchers and practitioners is whether complicated/prolonged grief should be included as a category of mental disorder in the next edition of the *DSM*. Given the significance of this issue, we devote two chapters in this book to consideration of the status of complicated grief as a category of mental disorder. In the first of these (chap. 8), Holly G. Prigerson, Lauren C. Vanderwerker, and Paul K. Maciejewski, strong proponents of introducing the new category, present a body of empirical research and argue the case that *prolonged grief disorder* (PGD; formerly known as *complicated grief*) constitutes a distinct mental disorder requiring inclusion in the *DSM* system. They elaborate on the features of PGD, identify specific risk factors for its occurrence, and describe possibilities for treatment. They also propose criteria for reliable and valid diagnosis of PGD and review trials they are conducting to assess the adequacy of the criteria for inclusion in the *DSM*.

There are good reasons to argue the case for inclusion of complicated grief/prolonged grief in the *DSM* system, not the least of which is to enable professional help to be given to those who most need it. However, there are also arguments against inclusion—for example, that it may unnecessarily medicalize grief or cause the natural family support network to withdraw if a grieving person is seen as needing and possibly receiving professional help. In chapter 9, then, Simon Shimshon Rubin, Ruth Malkinson, and Eliezer

Witztum discuss a number of challenges associated with DSM categorization of complicated grief. They draw on lessons learned from the classification of posttraumatic stress disorder in the planning for complicated grief, discuss the complications in conceptualization due to the interface of trauma and bereavement, and highlight difficulties associated with adopting a symptomatic phenomenological approach (e.g., assessment of the person's relationship with the deceased).

We have already argued the importance of expanding ethnic and cross-cultural research on bereavement, grief, and mourning. In chapter 10, Paul C. Rosenblatt addresses the need for cultural sensitivity, reviewing patterns of cultural influence and raising issues that all researchers need to be aware of. He advocates that for bereavement theory and research to advance, investigators must be sensitive to and informed by a cross-cultural perspective, including the belief that "culture creates, influences, shapes, limits, and defines grieving, sometimes profoundly" (p. 208). Attention to cultural perspectives in theory development in turn can enhance the provision of services and support to diverse grieving persons. Rosenblatt proposes an agenda for cross-cultural bereavement research in the future.

The concept of *disenfranchised grief* has had an enormous impact on the bereavement field, in large part because of the contributions of Kenneth J. Doka, author of chapter 11. The term denotes cases where grief has gone unrecognized, is marginalized, or has gone unsupported (e.g., in earlier times, loss of an out-of-wedlock partner; implications for mortality and ensuing bereavement because of positive DNA testing for Huntington's disease in contemporary society). From this viewpoint, then, a bereaved individual may be excluded from normative mourning and support processes, limiting his or her ability to acknowledge, experience, and adapt to loss. Doka describes the phenomenon of disenfranchised grief and its typologies, reviews research originating from different fields (e.g., psychological, sociological, and spiritual viewpoints), and considers complications for the grieving process and implications for counseling.

Another basic question is whether the concept of grief should be applied only to the loss of a loved one or close attachment figure. If so, what might be the meaning and significance of public demonstrations of grief that are seen with increasing frequency in the media, for example, for celebrities people have never met, or in response to traumatic events of national or international scope? In chapter 12, Tony Walter considers private and public mourning in the 20th century, introducing the concept of *new public mourning* (NPM)—the mourning for persons one did not know—and exploring its proliferation and transformation. He further examines the causes for this evolving phenomenon: Does the cause lie with the media, or does NPM have broader cultural, political, or psychological origins in changing society? Finally, he considers clinical and policy implications of NPM.

Patterns and Consequences of Grief: Phenomena and Manifestations

Since the publication of the last *Handbook* (M. Stroebe et al., 2001), important advances have been made with respect to understanding the experience and consequences of losing a loved one. This reflects scientific developments on many levels, from availability of prebereavement measurement in longitudinal studies and statistical advancements to the pinpointing of ways to establish neural correlates of emotion. These developments are extensively represented in Parts IV and V of this volume, which focus on patterns and consequences of grief. In Part IV, the chapter authors explore the course, manifestations, and diversity of grief reactions on both the psychological and physiological levels.

As in any other discipline, topics in bereavement go in and out of fashion, and the impact of the caregiving period before loss, including the possibility for anticipatory grief, has not been a popular research topic for some decades. Yet prebereavement experience is critically important. Richard Schulz, Kathrin Boerner, and Randy S. Hebert conducted a large-scale longitudinal investigation, which they describe in chapter 13, that followed the prebereavement caregiving of spouses and subsequent adaptation to bereavement. A main focus was on the impact of caregiving burden on subsequent adaptation. These authors also review existing research on the effects of bereavement when it is preceded by chronic illness, disability, and caregiving and describe their own research and the major findings, defining when and for whom a prebereavement period of preparation for death may be helpful versus harmful. They also suggest avenues for intervention and treatment.

In recent years, researchers have looked beyond stages or phases of grief through which bereaved persons were understood to proceed, postulating finely grained patterns of grieving. The availability of longitudinal data sets that include data collected before bereavement, to facilitate comparisons with patterns of adjustment after loss, has also enabled examination of trajectories of grieving and preloss predictors of different patterns. George A. Bonanno, Kathrin Boerner, and Camille B. Wortman, authors of chapter 14, have been at the forefront of this research. They have distinguished unique trajectories of bereavement outcome associated with different reactions to, and processing of, loss. In chapter 14, they provide a historical review and summarize their findings, enabling the reader to reassess such long-accepted notions as stages of grief, the role of denial, and the limits to resilience among the bereaved.

Whereas George A. Bonanno and his colleagues have greatly enhanced an understanding of resilience, Christopher G. Davis has made a major contribution with respect to the somewhat surprising phenomenon—given the devastating nature of loss—of personal growth during bereavement. In chapter 15, Davis describes his theoretical and empirical research on this topic. He views loss as a potential catalyst for change, as a challenge not only to the

self but also to one's goal system. Growth involves significant changes in life direction and life goals, which come about through processes of renegotiation, leading to the establishment of new, intrinsically meaningful goals and priorities. He argues for the need to distinguish *growth* from *benefits*, which is a new, more sophisticated way of looking at posttraumatic growth.

The research programs we have just described have contributed to an understanding of the kinds of coping strategies or cognitive mechanisms that enhance resilience and promote growth after bereavement. A remaining puzzle, however, concerns the role of meaning-making systems in general and of religious coping in particular. Do religious beliefs help a bereaved person to explain, accept, and come to terms with loss? In chapter 16, Judith C. Hays and Cristina C. Hendrix examine the role of religious meaning-making and religious coping strategies in bereavement. They detail the types of investigations of religious coping that are now available, surveying studies across different faith traditions and reviewing the major findings in relationship to adjustment, and they examine implications for practitioners and intervention researchers.

In previous *Handbooks* (M. Stroebe et al., 1993, 2001), considerable attention was given to physiological concomitants of grief (e.g., immune system changes, physiological changes associated with intervention). However, there have also been remarkable developments in the examination of brain systems in recent years. Functional magnetic resonance imaging of the brain is a relatively new but widely used technique to investigate the neural circuits associated with emotion–motivation systems, such as romantic attraction. In view of the similarity between feelings of romantic rejection and the emotions experienced after bereavement, John Archer and Helen Fisher present in chapter 17 recent psychobiological research involving the neural mechanisms underlying the former state and examine its possible implications for the psychobiology of bereavement. It becomes clear that research at this level can help to illuminate the nature and functions of grief and grieving.

Patterns and Consequences of Grief: Relationship Perspectives

Despite a long tradition of research on risk factors for bereavement outcome, it is still difficult to predict who among the bereaved is likely to suffer most and to be most in need of help. Furthermore, research on some vulnerable subgroups has been neglected, and thus society is ill informed of their special risks and requirements. We need to learn more about particular reactions and needs that follow specific types of loss. Part V of this volume, then, is devoted to emerging research on diverse subgroups of bereaved people, grieving under different circumstances. We have already highlighted the importance of focusing on the lost relationship in outlining the chapters (5 and 6) that focus on attachment theory and continuing bonds. We pick up this theme

in Part V, selecting four types of loss that represent contrasting relationships to deceased persons. The research described in these chapters suggests new insights regarding the dynamics of the grief and grieving of differing populations and how they may need special consideration both in research and practical support contexts.

The death of a child from any cause has been shown to have an extreme impact on parents, worse than any other type of bereavement. It has even been established that death of a child increases the mortality risk of mothers and fathers, particularly in the early weeks and months of bereavement (Li, Precht, Mortensen, & Olsen, 2003). Thus, researchers must continue to try to increase their understanding of parental bereavement. Of particular interest is the question of whether there might be even greater risk among parents of a child who has died from violent causes. With murder and suicide rates of young people increasing, answers to such key questions are urgently needed to enable appropriate interventions to be directed to parents who most require them. In chapter 18, Shirley A. Murphy describes her research program on parental bereavement specific to the violent subset of deaths (i.e., deaths due to accident, suicide, and homicide). To place these deaths in comparative perspective, she also summarizes the available research findings for parents whose children have died after an extended illness. She discusses difficult and sometimes controversial issues, for example, whether one violent cause of death of a child influences parents' outcomes more than another, or whether deaths due to suicide are particularly harrowing for parents.

There has been a long-standing debate in the scientific literature about the long-term effects on children of the death of a parent. Some investigators have found negative effects (into adulthood), whereas others have failed to confirm such connections. In chapter 19, Linda J. Luecken reviews evidence concerning both the short- and long-term impact of early loss of a parent on psychological and, of particular importance, on physiological outcomes (on which much new research is available). She examines alterations in the development of biochemical, hormonal, emotional, and behavioral responses to the environment and later life stressors. An important question here concerns why some children are resilient in the long term, whereas others appear much more vulnerable to the early loss event in their lives. Luecken examines social and environmental influences present both before and after the death, to establish moderators of the potential impact, including both risk and protective factors.

As life span expectancy has continued to increase over recent decades, issues surrounding the well-being and care of older adults are a cause for growing concern, especially as societies continue to struggle to provide adequately for their needs. A remarkable source of information about the bereavement experience among older persons has become available through the Changing Lives of Older Couples (CLOC) Study, a multiwave, large-scale prospective study of spousal bereavement. In chapter 20, Deborah Carr first reviews

historical, social, and psychological factors that shape the experiences of bereaved older spouses. She then provides a comprehensive overview of the CLOC Study, highlighting findings specific to late-life bereavement and reflecting on the contexts in which bereavement takes place at this time of life (e.g., not only factors associated with increasing frailty but also those associated with more effective use of [inter]personal resources). Implications for theory, practice, and future research are discussed.

Bereaved grandparents are a terribly neglected group in the bereavement literature, despite the risk that they may grieve deeply, not only for their deceased grandchild but also for their own child, the parent who is also grieving. Such a loss can be especially distressing at a time of life when the grandparent him- or herself may be frail. In chapter 21, Bert Hayslip Jr. and Diana L. White consider the experience and consequences of grief among grandparents over the loss of a grandchild. Two conceptual frameworks guide their analyses of the experience of loss among grandparents: (a) the life course perspective on intergenerational relationships and family systems and (b) disenfranchised grief. They compare the bereavement experience with the impact of divorce on relationships with grandchildren and the losses associated with raising a grandchild. They identify common elements across these different types of loss and illustrate how each contributes to our understanding of grandparent bereavement.

Development and Efficacy of Intervention Programs

Continuing controversies, generated in part by chapters in our previous *Handbook* (M. Stroebe et al., 2001), focus on the provision of professional and informal help for bereaved persons. A number of questions remain at the center of the debates: Is intervention necessary and useful at all? If it is, who benefits most? Can the grief process itself be eased through intervention? Which subgroups need help? What kinds of intervention programs are currently available? What sort of help has been shown to be efficacious? How can help be provided to the bereaved at times of massive trauma and human losses? In part VI, the last substantive section of this volume, the chapter authors provide up-to-date reviews on important intervention issues. The chapters in this part also cover evaluations of different types of intervention for different subgroups of bereaved persons.

Bereavement following disasters, whether on a relatively small or a colossal scale, is known to be associated with complications in the grieving process. Many factors compound the risk of adjustment difficulties (bodies missing or mutilated; children lost; witnessing horrific events; survivor guilt; additional losses of home, livelihood, etc.). How, then, can psychological care for bereaved persons best be provided by professional and voluntary services following such disasters? In chapter 22, Colin Murray Parkes draws on his own

considerable experience in organizing and providing care for bereaved people, from smaller scale, local disasters to international disasters. He carefully describes how the variability in the scope and magnitude of these disasters influences their impact and the nature of services necessary to cope with them. He also details other factors that influence the response, such as the duration of the event, cultural factors, and expectations of the bereaved individuals. He outlines the implications for bereavement practitioners and for community and national agencies of having to respond to a disaster.

A major advance in bereavement research has been the shift in focus from an intrapersonal perspective (on the grieving individual) to a broader, interpersonal perspective. After all, people do not grieve alone but among family and friends who may also be grieving, with mutual influence on the intensity and course of grief. This concern about including family and about group perspectives extends to therapy. Although in the past most interventions were designed to deal with a grieving person, there has been a more recent trend toward family-focused grief therapy. Given the importance of this broadened perspective, we include two chapters on family intervention. In chapter 23, David W. Kissane and Wendy G. Lichtenthal describe their family-focused grief therapy model and program. Family-focused grief therapy is a preventive intervention that begins during the terminal illness of a family member and continues beyond the person's death, intended to enhance family functioning and to encourage the sharing of grief among the family members. Kissane and Lichtenthal also describe controlled clinical trials that have assessed the therapy's efficacy. They place their own research within the perspective of other family-focused intervention programs, describing the principles underlying family approaches in general and the range of outcomes among different types of bereaved families.

In chapter 24, Janice Winchester Nadeau takes a very different type of family perspective. Nadeau focuses on interactive processes of family meaning construction, closely examining what goes on between and among family members during the time of bereavement. By what processes do families make sense of death? Nadeau addresses this basic question by drawing on her own research on family meaning-making and on her own considerable clinical experience. She integrates family theory, family therapy theory and methods, and relevant concepts from diverse scientific disciplines to provide ways to understand and appropriately intervene with families of the dying and bereaved. Again, in this chapter the need to link research with practice becomes evident: A family meaning-making model is presented that assumes that meanings attached to a death will affect bereavement outcomes in significant ways. Nadeau demonstrates how grief therapists might use the model and suggests guidelines for future research.

The research fields that have developed around bereavement and divorce have had little to do with each other, and there has been little cross-learning

between these differing areas of expertise. Yet comparisons between these two life experiences are of interest on both theoretical and practical levels, not only to elucidate comparative consequences (as addressed in chap. 21) but also to provide insight into effective intervention on family and individual levels. Irwin Sandler and his colleagues have conducted extensive empirical research with families who have experienced either bereavement or divorce. In chapter 25, they describe the development and evaluation of their theoretically derived Family Bereavement Program. The program was designed to prevent mental health problems in children who had experienced the death of a parent. First, they examine risk and protective factors among children who have lost a parent, and then they discuss modifiable factors that were targeted in their Family Bereavement Program. They provide evidence for the efficacy of the intervention in assisting children and adolescents through their loss experiences and include assessments of mediators and moderators of the program's effects. They discuss ways in which the findings with bereaved children replicate and diverge from those of their research with children in divorced families.

In chapter 26, the final chapter of Part VI, Margaret S. Stroebe, Karolijne van der Houwen, and Henk Schut take a critical look at how contemporary technological advances offer new possibilities for bereavement support. What is the potential, and what are the drawbacks, of using the Internet and e-mail systems for the giving and seeking of support among bereaved persons? First the authors describe recent expansions of available support through the Internet (e.g., online memorial sites, chat rooms, support groups, mailing lists for specific types of bereavement). They review possibilities for research and intervention via the Internet. They raise the question of the efficacy of using such sites and consider the issue of whether assessment actually needs to be made (and how this could be done). They draw attention to the paucity of research in this area, to the need for caution in assuming benefits of Internet bereavement support, and to potential harm that may be caused. A major purpose of the chapter is to set a research agenda for the future.

In chapter 27, this volume's closing chapter, we provide an overview, as we did in the last *Handbook* (M. Stroebe et al., 2001). We summarize and comment on the themes and chapters, pick up on the debates and controversies and give our own views on these, suggest additional topics that might need further attention, and propose lines of research for the future.

CONCLUSIONS

Our goal for this volume, then, is to present the ideas and research that have broadened and deepened our understanding of the bereavement experience over the past decade. Important assumptions have now been put to

empirical test, theoretical integration has progressed, and more sophisticated designs and methodologies and more advanced statistical techniques have been used and have provided new and more complex insights. In this context, important new controversies have arisen, causing much debate, and chapters in the volume address such issues from varying perspectives. New research has also shown how the bereavement experience itself has been changing in response to societal and cultural forces. Our hope is that this *Handbook* will provide a resource for bereavement researchers and practitioners alike, one that will be useful to them in their work with bereaved persons.

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